

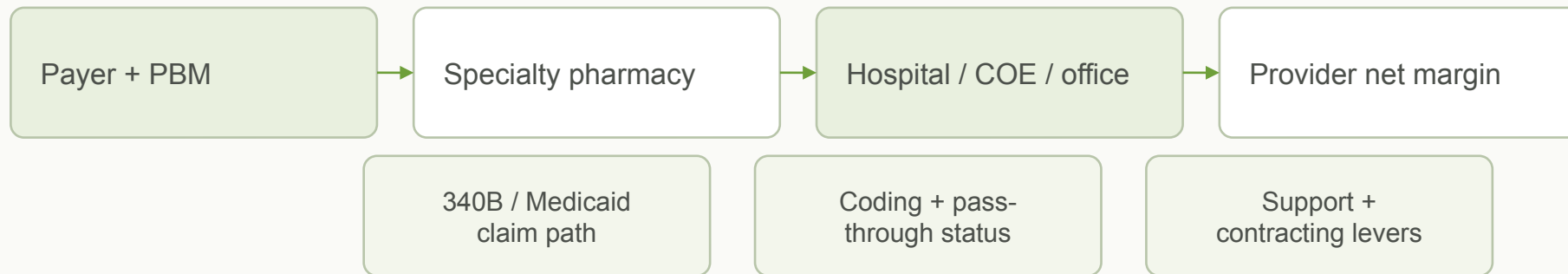
April 2026

Adrabetadex Provider Net Cost Calculator

Why launch uptake depends on provider economics, not clinical value alone

Prepared for the Beren Therapeutics launch team

Strategic internal discussion deck based on the concept brief



Provider economics sit between clinical interest and operational adoption.

The strategic issue

Clinical differentiation is necessary; provider economics still decide uptake.

- Buy-and-bill adoption depends on whether providers can recover acquisition, administration, and delay costs.
- White-bagging can eliminate drug spread and shift the provider equation to administration-only revenue.
- 340B creates a structural advantage for eligible centers of excellence over community sites.
- Medicaid coverage can still fail economically if PAD and NDC claim integrity break down.
- Coding and pass-through timing determine when hospital outpatient billing becomes predictable.

The calculator is a reimbursement execution model, not a list-price model.

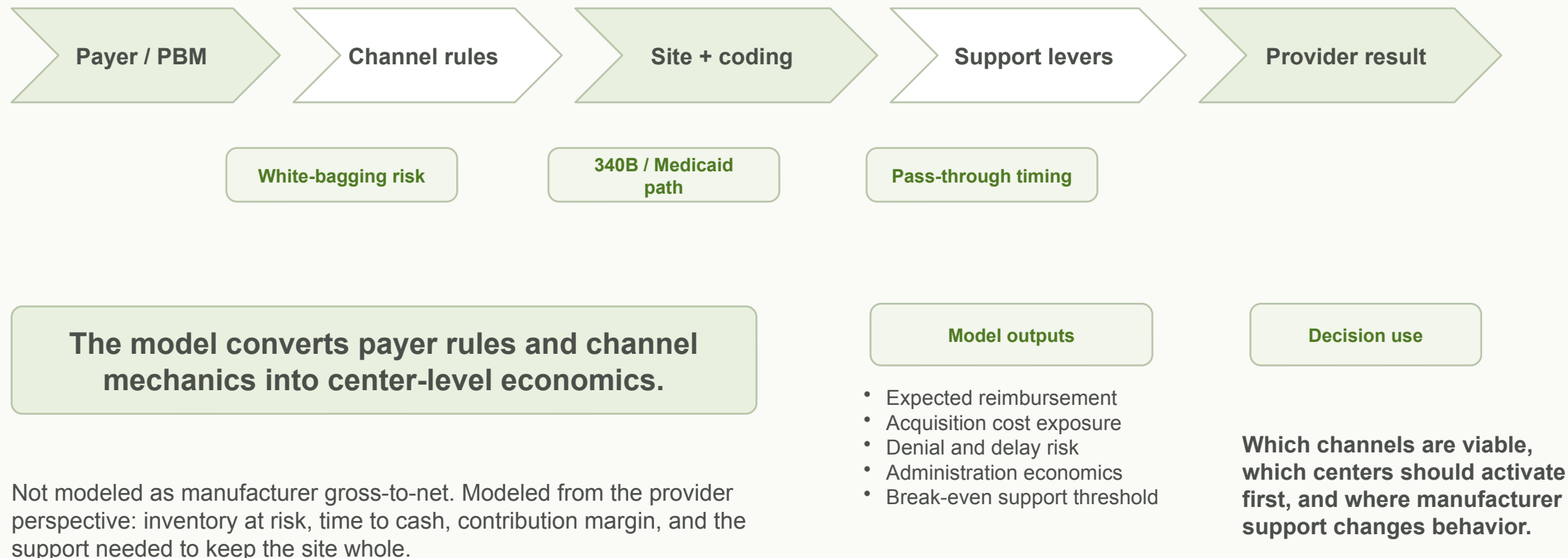
Launch question

Can a provider afford to buy, bill, and administer adrabetadex under real-world reimbursement conditions?

Provider-economics framing from the brief: uptake risk emerges when clinical interest reaches the point of stocking inventory.

What the model is designed to show

One workflow links reimbursement conditions to provider margin and working-capital exposure.



Why this matters for adrabetadex

The clinical profile may be attractive; the economic pathway is still fragile for providers.

Working launch assumption

\$39,000 per 900 mg vial
~\$1.014M annual gross drug exposure before administration cost

- High-cost therapy
- Likely hospital-centered pathway
- Ultra-rare pediatric setting
- Clinically differentiated profile
- Small claim failures can materially change site behavior

Financial fragility for the site of care

A few unpaid or delayed claims can make stocking and administering adrabetadex unattractive even when demand is clinically justified.

What makes the launch sensitive

Acquisition outlay arrives before clean reimbursement is proven.

Why the model matters

It shows where support, coding, and channel design change provider behavior.

Economic feasibility must be designed in before broad site activation.

The five launch variables that determine uptake

Each variable affects whether the provider can maintain a positive operating position.

1. Commercial medical benefit	PA status, specialist rules, site-of-care controls, white-bagging risk, discount sensitivity.	Shows whether buy-and-bill remains viable or is pushed toward near break-even.
2. Medicaid claim pathway	EPSDT tailwind, PAD methodology, NDC capture, FFS versus managed care split.	Distinguishes medical necessity from actual payment reliability.
3. 340B acquisition economics	Estimated ceiling-price advantage, carve-in or carve-out treatment, duplicate-discount controls.	Highlights which COEs are structurally advantaged at launch.
4. Outpatient coding and pass-through	Miscellaneous-code bridge, HCPCS timing, OPPS pass-through readiness.	Determines when hospital outpatient billing becomes predictable enough to scale.
5. Manufacturer support structure	Prompt-pay discount, acquisition rebate, free-drug bridge, patient-support assumptions.	Separates support that fixes provider economics from support that only reduces abandonment.

Provider economics vary sharply by channel

The scenario logic is provider-centered: buy, bill, administer, and collect.

Commercial buy-and-bill No meaningful support strained \$39,000 acquisition with reimbursement compression against WAC. Negative / near break-even Commercial access may exist, but denial drag and carrying cost can deter stocking.	Commercial buy-and-bill 5% support threshold Acquisition reduced by provider-facing support of about \$1,950 per dose. Near-neutral to modestly positive Represents the minimum realistic support threshold for non-340B participation.	Commercial buy-and-bill 10% support viable Acquisition reduced by provider-facing support of about \$3,900 per dose. Consistently positive Broadens adoption beyond a few high-margin centers and reduces stocking resistance.
Commercial white-bagging threshold Specialty pharmacy supplies drug; provider bills administration only. Drug spread removed Focus shifts to administration economics, clean claims, and operational simplicity.	Medicaid non-340B strained Coverage may exist, but payment depends on PAD data capture and state methodology. Weak participation likely Community-site economics stay unattractive without a cleaner payment path.	340B center of excellence advantaged Estimated acquisition advantage versus WAC creates structural margin support. Structurally advantaged Early uptake is most favorable in eligible hospital systems and COEs.

What the model suggests for launch strategy

The economic readout becomes a sequencing tool for launch operations.

1

Prioritize 340B-advantaged centers of excellence early.

Use structural margin advantage to anchor early adoption where the pathway is already favorable.

2

Identify PBM and specialty-pharmacy rules that can force white-bagging.

Treat sourcing rules as launch design inputs, not downstream contracting details.

3

Quantify the minimum provider support needed for non-340B uptake.

Separate provider-facing support from patient affordability support when setting launch budgets.

4

Build HCPCS and pass-through readiness into outpatient sequencing.

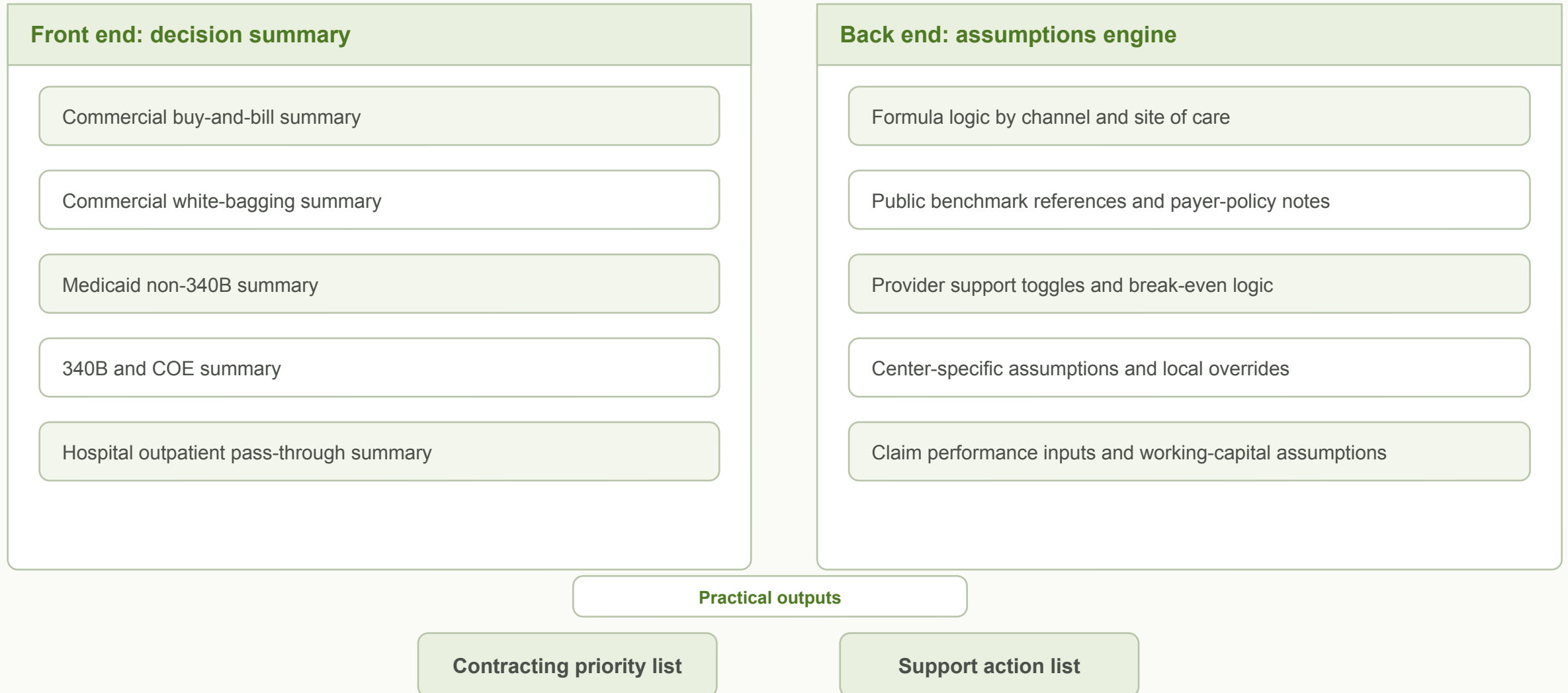
Plan hospital-outpatient readiness around code timing and the earliest practical pass-through window.

Final takeaway

Clinical differentiation may open doors, but reimbursement mechanics determine scalable site activation.

Recommended calculator format

Build a decision-facing front end with a transparent assumptions engine behind it.



Bottom line

The launch opportunity is real, but site-level economics will decide whether it scales.

1

Adrabetadex may have a compelling clinical story.

2

Provider uptake will still be determined by reimbursement execution and channel design.

3

Use the provider net cost calculator before launch to prioritize channels, centers, and support design.

The right reimbursement design can turn clinical interest into operational adoption.

Deck built from the concept brief only; no external facts added.